

SYNTHETIC / DEMO DATA

HARMONY GENERAL HOSPITAL

Hospital Policies and Procedures

Policy Manual — Edition 1.0

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SECTION 1

ICU Visitor Policy

This policy establishes the rules for visiting patients in the Intensive Care Unit (ICU) at Harmony General Hospital. The ICU cares for critically ill patients who require close monitoring, and visiting arrangements are designed to balance patient recovery, infection control, and the emotional support that family presence provides.

ICU Visiting Hours

- **ICU visiting hours:** 4:00 PM to 6:00 PM, daily

Visiting outside of these hours is not permitted except under the emergency exceptions described below. Nursing staff may briefly extend or interrupt visiting hours if patient care activities require the room to be clear.

Number of Visitors Allowed at One Time

- **Maximum ICU visitors at one time:** 2 visitors per patient

Only two visitors may be present at a patient's bedside at any given time. Additional family members may wait in the ICU family waiting area and rotate in as others step out.

Visitor Identification Requirements

- All ICU visitors must check in at the ICU reception desk before entering the unit.
- A valid government-issued photo ID is required for every visitor.
- Visitors will be issued a temporary ICU visitor badge that must be worn at all times while in the unit.
- The visitor badge must be returned to the reception desk when leaving the unit.

Children Visiting the ICU

- **Minimum age for ICU visitors:** 12 years old

Children under 12 years old are not permitted to visit the ICU, in order to protect both the child and the critically ill patients from infection risk. Children aged 12 to 17 must be accompanied by an adult at all times and may be asked to wait outside the room during certain procedures.

Exceptions for Emergencies

The ICU charge nurse may authorize exceptions to standard visiting hours and visitor limits in the following situations:

- The patient is in critical or end-of-life condition, as determined by the attending physician.

- A patient specifically requests the presence of a family member for comfort or decision-making support.
- Clergy or spiritual care providers are requested for last rites or spiritual support.

In these cases, the charge nurse may permit visitation outside the 4:00 PM–6:00 PM window and may temporarily allow more than two visitors at the bedside, at the discretion of the care team.

SECTION 2

General Visitor Policy

This policy applies to all non-ICU inpatient units at Harmony General Hospital, including medical-surgical floors, maternity, pediatrics, and rehabilitation units.

General Hospital Visiting Hours

- **General visiting hours:** 10:00 AM to 8:00 PM, daily

Patients may request quiet time or ask staff to limit visitors at any point during visiting hours. Individual units may post additional guidance near their entrance.

Visitor Registration

- All visitors must register at the main lobby visitor desk and receive a printed visitor badge.
- Visitor badges must be worn visibly at all times while inside the hospital.
- Visitors are asked to sign out at the visitor desk when leaving for the day.

Maximum Number of Visitors

- **Maximum general visitors per patient:** 3 visitors at one time

No more than three visitors may be present in a patient's room at the same time. This limit helps maintain a comfortable environment for the patient and their roommate, where applicable, and supports infection control practices.

Quiet Hours

- **Hospital quiet hours:** 9:00 PM to 6:00 AM

During quiet hours, hallway lighting is dimmed, overhead paging is limited to urgent announcements, and visitors are generally asked to conclude their visit unless they have been approved as an overnight companion by the nursing staff.

Visitor Behavior Requirements

- Visitors experiencing cough, fever, or other signs of illness are asked not to visit and to contact the nursing station instead.
- Food and beverages brought for patients must be approved by nursing staff, particularly where dietary restrictions apply.
- Visitors must follow all posted infection control and isolation signage before entering a patient's room.

- Disruptive, threatening, or abusive behavior toward staff, patients, or other visitors will result in removal from the premises.
- Photography or video recording of other patients, staff, or hospital equipment is not permitted.

SECTION 3

Patient Admission Policy

This policy describes the standard procedure for admitting patients to Harmony General Hospital, whether through scheduled admission, physician referral, or the Emergency Department.

Admission Procedure

- Patients or their representatives check in at the Admissions Office located on the first floor, near the main lobby.
- An admissions coordinator verifies the physician's admitting order and assigns the patient to the appropriate unit and bed.
- A member of the nursing staff completes an initial intake assessment, including vital signs, medical history review, and fall-risk screening, within 60 minutes of arrival on the unit.
- The patient is provided with an identification wristband, a copy of the Patient Rights and Responsibilities brochure, and an orientation to the room and call-button system.

Required Patient Information

To complete admission, the Admissions Office collects the following information:

- Full legal name, date of birth, and current address
- Emergency contact name and phone number
- Insurance or payment information
- Primary care physician and referring physician, if applicable
- Known allergies and current medication list
- Advance directive status, including any living will or healthcare power of attorney on file

Identification Requirements

- A valid government-issued photo ID is required for all non-emergency admissions.
- Patients unable to provide identification will be admitted under the hospital's Unidentified Patient Protocol, using a temporary identifier until identity is confirmed.
- An identification wristband with the patient's name, date of birth, and unique medical record number is applied immediately upon admission and is checked before every procedure, medication administration, or transfer.

Emergency Admissions

Patients admitted directly from the Emergency Department follow an expedited process: the Emergency Department physician enters the admitting order electronically, and the receiving unit is notified to prepare a bed. Identification and insurance information may be collected after the patient is stabilized, in accordance with the hospital's Emergency Medical Treatment obligations.

Documentation Requirements

- A signed Consent for Treatment form must be on file before non-emergency care begins.
- A signed Notice of Privacy Practices acknowledgment must be completed at admission.
- All admission documentation is entered into the hospital's electronic health record (EHR) within 24 hours of admission.

SECTION 4

Patient Discharge Policy

This policy ensures that patients leave Harmony General Hospital safely, with a clear understanding of their ongoing care needs.

Doctor's Discharge Approval

No patient may be discharged without a written discharge order from the attending physician. The physician documents that the patient's condition is stable and appropriate for discharge, and specifies the discharge destination (home, skilled nursing facility, rehabilitation center, or other setting).

Medication Instructions

- A discharge nurse reviews all new and continuing medications with the patient and, where applicable, a family caregiver.
- Written medication instructions include drug name, dosage, frequency, purpose, and possible side effects.
- Patients are given a reconciled medication list showing which prior medications are continued, changed, or stopped.
- The pharmacy team is available to answer medication questions before the patient leaves the unit.

Follow-Up Appointments

Before discharge, the care team schedules or confirms necessary follow-up appointments, which may include a primary care visit, specialist consultation, or lab work.

- **Standard follow-up appointment window:** within 7 to 10 days of discharge

Follow-up appointment details are printed on the discharge summary, including the provider name, location, date, and time.

Discharge Documents

Every discharged patient receives a Discharge Summary packet containing:

- A summary of the hospital stay and treatment provided
- Reconciled medication list and prescriptions
- Follow-up appointment details
- Warning signs and symptoms that should prompt a return to the Emergency Department
- Contact information for the discharging unit and primary care provider

Transportation Requirements

Patients must arrange safe transportation home prior to discharge. Patients who received sedation, certain pain medications, or anesthesia within the prior 24 hours may not drive themselves and must be accompanied by a responsible adult. Case management can assist patients who lack transportation by coordinating approved medical transport services.

SECTION 5

Emergency Department Policy

The Emergency Department (ED) at Harmony General Hospital is open 24 hours a day, 7 days a week, and treats patients based on the severity of their condition rather than order of arrival.

Emergency Triage

All patients arriving at the Emergency Department are assessed by a triage nurse within 10 minutes of arrival using a standardized 5-level triage scale.

- **Target time to initial triage assessment:** within 10 minutes of arrival
- Level 1 (Resuscitation): immediate, life-threatening condition
- Level 2 (Emergent): high-risk condition requiring rapid treatment
- Level 3 (Urgent): stable but requires timely evaluation
- Level 4 (Less Urgent): stable, minor condition
- Level 5 (Non-Urgent): stable, minimal resources needed

Priority of Treatment

Patients are treated according to their triage level and clinical need, not their arrival time. A patient with a less severe condition may wait longer if higher-acuity patients arrive after them. Triage nurses reassess waiting patients periodically and will escalate a patient's priority level if their condition changes.

Emergency Contact Procedures

- Patients or accompanying family members are asked to provide an emergency contact at check-in.
- For patients who arrive unaccompanied and unable to communicate, staff will attempt to identify and contact next of kin using available identification and hospital records.
- Law enforcement or emergency medical services personnel who transported the patient are asked to provide any known contact information.

Waiting-Room Guidelines

- Patients and companions should remain in the waiting area until called by name or number.
- One companion may generally accompany a patient into the treatment area, space permitting; additional companions may be asked to remain in the waiting room.
- Waiting room visitors are asked to notify staff of any worsening symptoms immediately.
- Food, beverages, and personal belongings should be kept with the visitor at all times.

SECTION 6

Medication Policy

This policy governs how medications are verified, administered, documented, and monitored across Harmony General Hospital to protect patient safety.

Medication Verification

Before any medication is administered, nursing staff perform the 'Five Rights' check:

- Right patient — confirmed using two identifiers (name and date of birth)
- Right medication
- Right dose
- Right route
- Right time

Medication orders are verified against the pharmacy-reviewed electronic medication administration record (eMAR) prior to administration.

Medication Administration

- Patient identity is confirmed by scanning the patient's wristband barcode against the medication barcode before administration.
- High-alert medications (such as insulin, anticoagulants, and opioids) require independent double-checking by a second licensed nurse.
- Any missed or delayed dose is documented in the eMAR with the reason and reported to the attending physician if clinically significant.

Patient Medication Records

Each patient's complete medication history, including home medications, in-hospital orders, and discharge prescriptions, is maintained in the electronic health record. Medication reconciliation is performed at every transition of care: admission, transfer between units, and discharge.

Allergy Verification

- Allergy status is confirmed with the patient (or family member/caregiver) at admission and documented prominently in the electronic health record.
- A red allergy wristband is applied to any patient with a known allergy.
- The pharmacy system automatically screens new orders against documented allergies and flags potential conflicts before dispensing.

Reporting Medication Errors

Any suspected medication error, near-miss, or adverse drug reaction must be reported immediately to the charge nurse and the attending physician, and documented in the hospital's electronic incident reporting system within the same shift. The Pharmacy and Therapeutics Committee reviews reported medication events monthly to identify trends and prevention opportunities.

SECTION 7

Infection Control Policy

Harmony General Hospital follows evidence-based infection prevention practices to protect patients, staff, and visitors.

Hand Hygiene

- All staff, patients, and visitors must perform hand hygiene using alcohol-based hand sanitizer or soap and water before and after any patient contact.
- Hand hygiene stations are located at every room entrance and throughout common areas.
- Staff must perform hand hygiene before donning and after removing gloves.

Personal Protective Equipment (PPE)

- Gloves, gowns, masks, and eye protection are provided at the entrance of any room requiring isolation precautions.
- PPE must be put on before entering an isolation room and removed in the correct sequence before exiting, followed immediately by hand hygiene.
- Single-use PPE items are discarded after each patient encounter and never reused.

Isolation Procedures

Patients with a suspected or confirmed contagious condition are placed on the appropriate isolation precaution level, indicated by color-coded signage on the door:

- Contact Precautions (yellow sign): gown and gloves required for room entry
- Droplet Precautions (blue sign): surgical mask required within 3 feet of the patient
- Airborne Precautions (red sign): fitted N95 respirator required; patient placed in a negative-pressure room

Isolation signage and precaution level are reviewed by the infection control team at least every 72 hours or when the patient's clinical status changes.

Infection Reporting

Suspected hospital-acquired infections must be reported to the Infection Prevention and Control Department within 24 hours of identification. The department tracks infection rates by unit and reports trends to hospital leadership on a monthly basis, escalating any outbreak concerns immediately.

Visitor Precautions

- Visitors to isolation rooms must follow the same PPE requirements as staff and check with the nursing station before entering.
- Visitors with symptoms of a contagious illness (fever, cough, vomiting, diarrhea, or rash) are asked to postpone their visit.
- During declared outbreak periods, the hospital may temporarily restrict visitation to affected units; such restrictions are posted at unit entrances and communicated to families directly.

SECTION 8

Patient Privacy Policy

Harmony General Hospital is committed to protecting the confidentiality of patient health information in accordance with applicable privacy laws and hospital standards.

Protection of Patient Information

- Patient health information is stored in the secure electronic health record system, which requires individual staff login credentials and maintains an access audit log.
- Printed or physical documents containing patient information must be kept in secure areas and shredded when no longer needed.
- Discussions of patient information should take place in private areas, away from public hallways, elevators, and waiting rooms.

Who May Access Patient Records

- Members of the patient's direct care team, including physicians, nurses, and allied health staff involved in the patient's treatment
- Hospital departments requiring access for billing, quality review, or regulatory compliance purposes
- The patient, or their legally authorized representative, upon request through the Health Information Management (HIM) Department

Access to records is granted on a minimum-necessary basis: staff may view only the information required to perform their specific role.

Sharing Information with Authorized Individuals

- Information may be shared with family members or friends only if the patient has given verbal or written permission, or if the patient is incapacitated and disclosure is determined to be in their best interest.
- Patients may designate specific individuals authorized to receive updates using the Patient Communication Preferences form completed at admission.
- Requests for records from outside providers, insurers, or legal representatives must be processed through the HIM Department with proper authorization on file.

Confidentiality Requirements

- All staff complete privacy and confidentiality training upon hire and annually thereafter.

- Staff may not access records of patients they are not directly caring for, including friends, family members, or coworkers, regardless of curiosity or personal connection.
- Any suspected privacy breach must be reported immediately to the Privacy Officer for investigation.

SECTION 9

Staff Conduct Policy

All employees, contractors, and volunteers at Harmony General Hospital are expected to uphold the highest standards of professionalism, safety, and respect.

Professional Behavior

- Staff must wear hospital-issued identification badges at all times while on duty.
- Staff are expected to arrive on time, follow the dress code for their role, and maintain a clean, professional appearance.
- Staff must treat all patients, families, and colleagues with courtesy and respect, regardless of background or circumstances.

Patient Communication

- Staff must communicate with patients using clear, respectful, and age-appropriate language, avoiding unnecessary medical jargon.
- Staff should introduce themselves by name and role at the start of every patient interaction.
- Patients and families should be given the opportunity to ask questions, and staff should verify understanding before ending important conversations.
- Interpreter services must be offered to any patient or family member with limited English proficiency or a hearing impairment.

Confidentiality

Staff must protect patient information at all times, as detailed in the Patient Privacy Policy (Section 8), including refraining from discussing patients by name in public areas or on personal social media.

Workplace Safety

- Staff must follow all infection control, PPE, and hand hygiene requirements described in Section 7.
- Any workplace injury, needle-stick, or safety hazard must be reported to a supervisor and the Environmental Health and Safety office immediately.
- Staff must complete annual safety training, including fire safety, emergency codes, and workplace violence prevention.
- Harassment, discrimination, or violence of any kind toward staff, patients, or visitors will not be tolerated and will be addressed through Human Resources.

SECTION 10

Contact and Escalation Procedure

This policy outlines who staff should contact for various situations, ensuring issues are addressed by the appropriate person quickly and consistently.

Who Staff Should Contact for Different Issues

- Clinical questions about a specific patient: the attending physician or the unit charge nurse
- Medication concerns or drug interaction questions: the on-duty pharmacist
- Visitor policy disputes or behavioral concerns: the unit charge nurse, escalating to hospital Security if needed
- IT or electronic health record issues: the IT Service Desk
- Facilities issues (temperature, equipment malfunction, spills): the Facilities/Engineering Department
- Privacy or confidentiality concerns: the Privacy Officer

Emergency Escalation

For any immediate life-threatening emergency involving a patient, staff, or visitor, personnel must activate the appropriate hospital emergency code overhead and contact the Rapid Response Team without delay.

- **Rapid Response Team target response time:** within 5 minutes of activation
- Code Blue: adult or pediatric medical emergency / cardiac or respiratory arrest
- Code Red: fire
- Code Silver: weapon or hostage situation
- Code Grey: combative or violent person

Any staff member may activate an emergency code using the nearest code button or by calling the hospital operator and stating the code and location clearly.

Supervisor Notification

- Staff must notify their direct supervisor or charge nurse of any significant clinical change, safety concern, or unresolved conflict during their shift.
- Supervisors are expected to be reachable by hospital phone or pager at all times during their scheduled shift.
- If a supervisor is unavailable, staff should contact the House Administrator on duty, available 24 hours a day.

Incident Reporting

All incidents involving patient safety, medication errors, workplace injuries, security concerns, or privacy breaches must be documented in the hospital's electronic incident reporting system within 24 hours of the event.

- **Incident report submission deadline:** within 24 hours of the event

The relevant department director reviews all incident reports weekly, and serious events are escalated immediately to the Quality and Patient Safety Committee for further review.

End of Document — Harmony General Hospital Policy Manual (Synthetic/Demo Data, Edition 1.0)